

COLONOSCOPY REPORT (OUTSIDE RECORD, 12/18/2025)

Keystone Endoscopy Center – 850 Market St, Philadelphia PA

Patient: Cole, Jasmine DOB 02/14/1997 Procedure date: 12/18/2025

Endoscopist: P. Ramanathan, MD Indication: Postoperative Crohn's surveillance, s/p ileocecal resection 08/2024. Prep: Good (BBPS 8).

EXTENT: Cecal pouch/anastomosis reached; neo-terminal ileum intubated to 10 cm.

FINDINGS:

Ileocolic anastomosis: Multiple aphthous ulcers plus several larger ulcers (>5 mm) involving the anastomotic ring and extending into the neo-terminal ileum. Mild luminal narrowing, scope traversed easily. Neo-terminal ileum: Patchy erythema, aphthae, and 3 larger ulcers within the distal 10 cm. Colon: Normal mucosa throughout. Rectum: Normal.

IMPRESSION: Endoscopic postoperative recurrence – RUTGEERTS i2b (lesions confined to anastomosis plus >5 aphthous ulcers in the neo-terminal ileum with larger ulcers).

BIOPSIES: Anastomosis and neo-terminal ileum – chronic active ileitis, no dysplasia, no granulomata, CMV immunostain negative.

RECOMMENDATIONS: Optimize medical therapy (check anti-TNF level and antibodies; escalate or switch class per levels). Repeat ileo-colonoscopy in 6-12 months. Avoid NSAIDs. Report faxed to GI office.